

User Research

Team Ctrl+Agen — KADO Hackathon

1 User Research and Practical Validation

The team conducted targeted discovery interviews with four practitioners: **Rutuparna Kulkarni**, Clinical Research Coordinator at Heartcare Clinical Research LLP (Ahmedabad); **Dr. Surbhi Narula**, Ophthalmologist, GS Medical College & Hospital, Hapur; **Dr. Chetna Malik**, Dermatologist, GS Medical College & Hospital, Hapur; **Dr. Shubham Arora**, Radiologist, GS Medical College & Hospital, Hapur; **Aryana Nayak**, Intern, KIMS KOPPAL Medical College, Koppal, Karnataka; **Dr. Darshan D**, PharmD Graduate with experience in Clinical Research and Pharmacovigilance; and **Nishchita Gowda**, Postgraduate Student, KIMS KOPPAL Medical College, Koppal, Karnataka. Three recurring findings directly shaped the product.

- First, respondents described protocol rules and patient safety as inputs that are considered together in practice but are rarely presented together at the exact moment a decision is made.
- Second, they supported a **Modify** option because real clinical decisions are not always binary; they also warned that verbose or low-value alerts risk being ignored like routine clinical alerts. This led to concise, page-cited rationales rather than long free-form explanations.
- Third, respondents valued a lightweight audit trail for explaining deliberate overrides during later review, but emphasized that documentation must not create a new administrative burden.

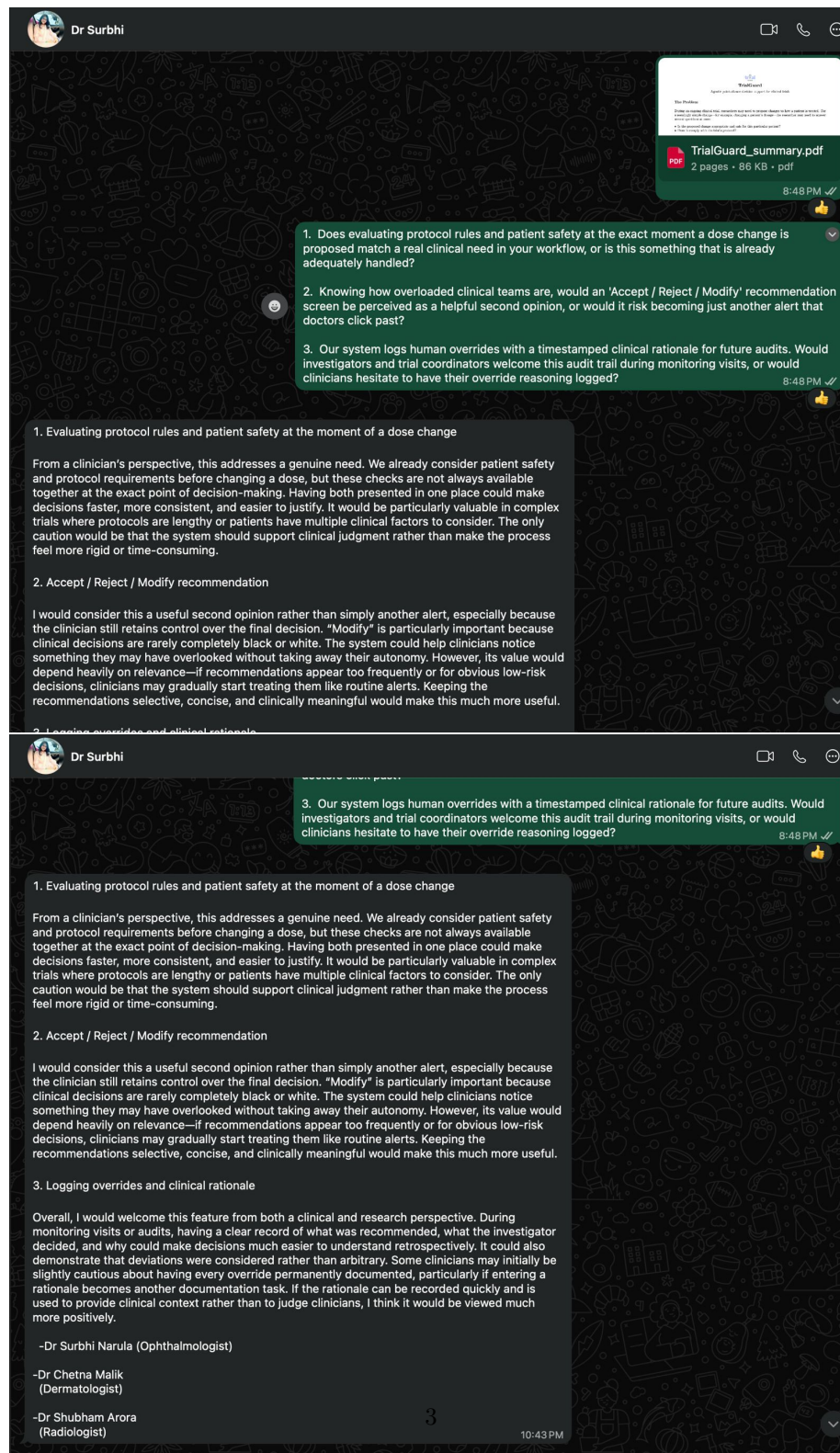


Figure 2: Dr. Surbhi Narula, Dr. Chetna Malik and Dr. Shubham Arora's Feedback

Dr. Darshan D

Clinical Research Coordinator (CRC) | PharmD | Clinical Tr...

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You haven't connected with Dr. Darshan D

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TUESDAY

Pranjal Kukreja

8:46 PM

Quick question for a hackathon on Clinical Trial Screening

Hi Dr.Darshan Sir,

I'm a computer science student at IIT Patna building a prototype for a hackathon. We're designing an AI system (TrialGuard) that automatically checks if a patient's EHR data matches the complex inclusion/exclusion criteria of a clinical trial protocol.

We are trying to understand the real-world pain points before our deadline on the 17th. If you have 2 minutes, I'd deeply appreciate your thoughts on these 3 quick questions:

- When screening a patient against a protocol, what is the most time-consuming part (e.g., finding the

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You haven't connected with Dr. Darshan D

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- When screening a patient against a protocol, what is the most time-consuming part (e.g., finding the right lab value, interpreting unstructured doctor notes)?
- How often do protocol deviations happen because a rule was accidentally missed during manual review?
- If an AI flagged that a patient was 'ineligible' for a trial, what evidence (e.g., citation to the PDF, raw lab value) would you need to trust its decision?

Even a few sentences via reply would be incredibly helpful for our project. Thank you so much!

Dr. Darshan D

9:45 PM

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- 2.How often do protocol deviations happen because a rule was accidentally missed during manual review?
- 3.If an AI flagged that a patient was 'ineligible' for a trial, what evidence (e.g., citation to the PDF, raw lab value) would you need to trust its decision?

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Dr. Darshan D

9:45 PM

1- Finding the right lab parameters that meet the inclusion criteria to prevent screen failure

2- The most common PD is when the patient is not able to visit the site as per the scheduled visits

3- raw lab value

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Figure 3: Dr. Darshan D's Feedback